

India's Population Programme

Obstacles and Opportunities

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As sterilisation scandals abound, a consensus is emerging for a shift away from sterilisation towards a larger “basket” of contraceptive choices and concomitant improvements in service delivery. That such a shift needs to take place is clear, but precisely how it is to come about, and who gets to determine what is in the basket of choices are questions that deserve greater attention. The neo-Malthusian resurgence, combined with the technical fixation on contraception, favours certain methods over others, but health and safety concerns related to these methods are typically downplayed or suppressed.

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In November 2014, India was rocked by yet another sterilisation scandal that made international headlines. Thirteen women died after being sterilised in a camp in Bilaspur District, Chhattisgarh. Fact-finding missions by women's health and human rights organisations revealed appalling conditions at the camp (Population Foundation of India et al 2014; Sama Resource Group for Women and Health et al 2014). The Chhattisgarh tragedy is the tip of a much larger iceberg. India still relies overwhelmingly on female sterilisation in its family planning programme, and states are essentially given free rein to impose sterilisation targets—now euphemistically called Expected Levels of Achievement. Quality of care for all methods is abysmal, with family planning providers receiving minimal training, and no supervision.

Periodically, winds of change sweep through at the policy level. In 1996, the government officially adopted a target-free approach, reflecting the shift from population control towards reproductive health at the 1994 International Conference on Population and Development (ICPD) in Cairo. In 2000, it launched a new National Population Policy (NPP) based on free and informed consent, critical of coercion. Yet, rhetoric has not translated into action.

Today, it appears that winds of change are blowing again. In more liberal family planning quarters, a consensus is emerging for a shift away from sterilisation towards a larger “basket” of contraceptive choices and concomitant improvements in service delivery. There is an implicit recognition that the demographic transition to low birth rates is well under way in India, and the real issue at hand is not pressurising women to stop having children, but meeting their demand for safe contraception

as part of broader reproductive health services.

That such a shift needs to take place is clear, but precisely how it is to come about, and who gets to determine what is in the basket of choices, are questions that deserve greater attention. Here, we examine the larger political context in which efforts to reform the Indian population programme are taking place, starting at the international level.

The Neo-Malthusian Resurgence

Unfortunately, the influence of the international population field today may be less conducive to the reform of the Indian population programme, than it was in the aftermath of the ICPD. Despite declining birth rates worldwide, neo-Malthusian ideology is resurgent on the global development stage. Within the population field, a shift in the balance of power has occurred as the old guard devotees of population control reassert their dominance over more progressive sexual and reproductive health and rights (SRHR) advocates.

The old guard has found an important new ally in the Bill & Melinda Gates Foundation (BMGF). In 2012, at the London Summit on Family Planning, a core partnership of the BMGF, the UK's Department for International Development (DFID), the United Nations Population Fund (UNFPA), and the US Agency for International Development (USAID) launched the Family Planning 2020 (FP2020) initiative, which aims to get 120 million more women and girls to use contraception by the year 2020 (“About Us,” Family Planning 2020). The FP2020 advocates are careful to explain that the 120 million figure is a goal, not a target.¹ Semantics aside, India aims to get 48 million more women to use contraception by 2020, and has already doubled incentive payments for sterilisation as a way of achieving its target (Kazmin 2014).

The FP2020's narrow focus on female contraception has come under criticism for undermining a broader SRHR agenda, neglecting men's roles in contraception, ignoring quality of care concerns, and sidelining the need for investments in

safe abortion services (Jain and Rama Rao 2015; Fried and Hendrixson 2014). Even within the FP2020's own ranks, SRHR-oriented members worry that the initiative could encourage coercive population control measures.²

The top-down, neo-Malthusian prerogatives of the FP2020 mesh well with other features of today's international development industry, including its managerial ethos, obsession with measurable outcomes and technical fixes, and the privileging of the private sector. Add to this the immense power of wealthy philanthropic interests, such as the BMGF, that wed technocracy with plutocracy (Birn 2014). Questions of power and political economy—in particular, neo-liberal assaults on social welfare—are conspicuously absent from policy discourse, replaced with a wishful win-win, public–private mantra.

In the population field, this win-win mantra takes the form of assuming that contraceptives produced by big pharma and distributed either through population programmes or social marketing schemes are the key to liberating poor women, spurring economic growth, and even saving the planet from ecological doom (Sasser 2011). These assumptions preclude a real discussion of which methods are the safest for women and men.

Who Is Calling the Shots?

In theory, India's "basket of choices" now contains sterilisation, intrauterine contraceptive devices (IUCDs), pills, condoms and abortion, though poor women face formidable obstacles in getting access to reversible contraceptives and safe, legal abortion. Expanding the basket of choices raises two main questions. First, what methods should be added to the basket? Second, how can the family planning programme, broken as it is, ensure access to even more methods and at the same time ensure quality of care?

The international setting matters here as well. The neo-Malthusian resurgence, combined with the technical fixation on contraception, favours certain methods over others, namely, those that are the most effective and cost-efficient in preventing pregnancy, and hence reducing birth rates. Implicit in this calculus is the

desire to minimise contraceptive failure, and hence the need for safe abortion. This is another consequence of the BMGF's entry into the field since Melinda Gates does not support abortion rights (Fried and Hendrixson 2014). The most favoured contraceptives are the three-month injectable Depo-Provera, and long-acting, reversible contraceptives such as IUCDs and hormonal implants. Health and safety concerns related to these methods are typically downplayed or suppressed.

Depo-Provera is a case in point. At the 2012 London Summit on Family Planning, a public–private partnership of the BMGF, USAID, DFID, UNFPA, the Pfizer pharmaceutical company, and the medical non-profit PATH announced a new initiative to reach 3 million women in sub-Saharan Africa and South Asia over the next three-year period, with 12 million doses of a new subcutaneous delivery form of Depo-Provera called Sayana Press (PATH 2012). From the standpoint of protecting public health, this mass promotion of Depo-Provera is highly questionable.

Depo-Provera has long been the subject of considerable medical controversy. Its known side effects include prolonged and irregular bleeding, suppression of immune response, loss of bone mineral density in young women, significant weight gain, depression, and loss of libido. There are clear racial disparities in its use, both in the US, where it is mainly promoted to women of colour, and globally, where sub-Saharan Africa is a primary target (USAID 2012; Volscho 2011). For over a decade now, medical studies have provided compelling evidence that Depo-Provera increases the risk of women and their partners becoming infected with HIV. A growing number of contraceptive researchers and epidemiologists are now questioning the wisdom of Depo-Provera's continuing promotion, especially in populations at high risk of HIV.³

International pressure is mounting on India to accept Depo-Provera into the national family planning programme. In addition to a BMGF-financed pilot project to introduce Depo-Provera into the public system in two districts in Haryana, the

Ministry of Health and Family Welfare has recently announced that it has approved in principle the method's inclusion in the basket of choices (*Indian Express* 2015; "Pathfinder International," Bill & Melinda Gates Foundation). To date, studies of Indian women's experiences with Depo-Provera reveal high discontinuation rates due to side effects, especially menstrual disturbances such as intermittent or heavy bleeding, which interfere with women's ability to do even routine household tasks. Moreover, the quality of information and counselling given about the method is poor (Jejeebhoy and Xavier 2012; Sama 2003).

Nevertheless, key leaders in non-governmental organisation family planning circles express enthusiasm for introducing Depo-Provera into the national programme. While acknowledging the problem of high discontinuation rates, they justify their support on the basis that poor women, especially from the Muslim community, like the method because they can keep it a secret from their husbands. However, side effects caused by Depo-Provera can actually make it difficult to hide one's use of the method. There is also a remarkable lack of awareness in family planning circles about the serious evidence linking Depo-Provera with a higher risk of contracting HIV.

Transforming the System

What are the main impediments to overhauling the family planning programme so that it could ethically and effectively provide a range of contraceptives, safe abortion, and other reproductive health services? First and foremost is the government's lack of commitment to public


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health in general. India spends just a little over 1% of its gross domestic product (GDP) on public health, one of the lowest figures in the world. Resource scarcities and inefficiencies, coupled with poor training and support of staff, plague multiple parts of the system, not just the family planning programme. Moreover, the present government is committed to further privatisation of healthcare delivery; public health is low on the list of priorities (Qadeer 2014).

Second is the powerful role neo-Malthusian ideology plays in both shaping national values and serving elite political interests. Quality of care improvements in family planning not only depends on more resources, but a fundamental shift in those values. The synergies between population control targets and incentives, contemptuous misogyny, and caste and class prejudices create fertile conditions for the sacrifice of poor women's health and lives in the name of saving the nation from overpopulation. Changing those values, and associated practices, is no mean feat.

In the face of these obstacles, SRHR advocates have fought long and hard to reform the family planning programme and retrain providers, from doctors to community workers (Khanna and Ravindran 2015). Abortion rights advocates are also working to make safe, legal abortion widely available. However, the tentacles of population control reach far beyond the family planning clinic or sterilisation camp. They are intricately bound up with the nationalist imaginary, and caste and religious politics, too useful and productive to be given up easily by the rich and the powerful. Through the imposition of the two-child norm at the state level, the population issue is deployed to disenfranchise and dispossess poor and marginalised communities. The two-child norm is only nominally about reducing population growth; after all, most states in India have reached or are approaching replacement-level fertility. Instead, it is a political tool to prevent the "wrong" sort of people from seeking local office or accessing public resources and benefit schemes (Majumdar 2013; Sangari 2015). Rajasthan, which, along with Odisha

and Andhra Pradesh, has imposed one of the most repressive forms of the two-child norm, has recently extended it to applicants for junior teacher positions.⁴

So far, the centre is content to leave most of the two-child norm's dirty work to the states, though, in almost every parliamentary session, bills are proposed that advocate for such a national norm. In the words of a former senior government official who has fought the norm for many years, "eternal vigilance" is required to block these bills.⁵

Rewriting History

The Indian population control programme would be even worse if it were not for the sustained and courageous efforts of feminist, health and human rights activists to keep it in check. The Indian public health and women's movements, in particular, have taken a leading role in exposing sterilisation abuse, pushing for contraceptive safety, and demanding systemic reforms. Yet, in recent years, on both national and international levels, the strength and independence of such movements have weakened. In the case of India, women's groups have had to take on such a host of serious challenges—from sex selection, to gender violence, to new reproductive technologies, to unethical drug trials—that the focus on population policy has diminished.

This reduced focus, while understandable, is problematic, especially at a time when neo-Malthusian interests have the upper hand internationally. History is also being rewritten to blame the Indian women's movement for the narrow range of contraceptives now available. Critics are painting the movement with the broad brush of being "anti-technology" because of its previous advocacy to keep injectables out of the national family planning programme. That there were real health and safety concerns about these methods then, as there are now, is being swept under the rug. A danger exists that the rich history of women's activism against population control, including the strategies used and lessons learnt, will be caricatured or even lost as a result (Rao and Hodges 2014).⁶

In the days ahead, it will be critical for feminist, public health and human rights activists to play the watchdog, as well as to engage in the political spaces where changes to Indian population policy and the Indian family planning programme are proposed and debated. The stakes are high.

NOTES

- 1 Observation based on interviews with family planning and sexual and reproductive health and rights (SRHR) professionals by Betsy Hartmann in New Delhi in spring 2015.
- 2 Observation based on interviews with family planning and SRHR professionals by Betsy Hartmann in New Delhi in spring 2015.
- 3 See "Citizen Petition from Heidi Jones" (2015). This petition has a list of recent medical journal articles on studies about Depo-Provera and HIV risk.
- 4 The Hunger Project and the Centre for Health and Social Justice in New Delhi are currently the organisations most closely monitoring the two-child norm.
- 5 Interview by Betsy Hartmann in New Delhi in spring 2015.
- 6 Observations based on interviews by both authors.

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